

ENGLISH FOR WORK / SEPTEMBER 2026

Pharmaceutical English



Conversation lab

12 complete workplace dialogues, followed by eight additional role-play cases. Study the exchanges, then make the conversation your own.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Study complete conversations. Find the right language, speak, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For clinical development, regulatory affairs, pharmacovigilance, quality, CMC, manufacturing, medical affairs, market access, and compliance teams.

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

Inside this conversation lab

Original fictional training conversations in professional English. These are realistic scripts, not transcripts of actual people. The professional roles are the speaker labels; all figures, incidents, and organizations are invented.

01 TPP Debate: Scientifically Interesting vs Clinically Meaningful

02 IND Readiness: CMC Is Not Just a Detail

03 Endpoint Debate: Convenient vs Clinically Relevant

04 Enrollment Rescue: Protocol Burden and Site Reality

05 Safety Signal: Possible Liver Risk

06 Data Readout: Statistically Significant but Modest Effect

07 Quality Deviation: Batch Release Pressure

08 Promotional Claim Review: Subgroup Data

09 MSL Boundary: Unsolicited Off-Label Question

10 Payer Evidence: Value Story Is Not the Same as Label

11 Biosimilar Discussion: Similar Is Not Identical

12 Launch Readiness: Approval Is Not Enough

Then try eight independent role-play cases, followed by feedback criteria and references.

Read it. Hear it. Make it yours.

1. Read the setting. Identify the roles, the immediate problem, and what each speaker needs.
2. Read the whole conversation aloud with a partner. In a group, give a third person the observer role. For three-speaker scripts, assign each professional a separate voice.
3. Notice how the speakers ask a specific question, qualify a claim, disagree, explain a technical point, or agree on a next step. Underline language you would actually use.
4. Cover the script. Recreate the exchange using the situation and professional roles. Keep the meaning, but change the wording.
5. Switch roles and introduce a complication: a missing record, a different priority, an unavailable colleague, or a changed assumption. End with a clear outcome or unresolved question.

Register and terminology

These colleagues use concise professional language. In an internal technical meeting, familiar abbreviations can be natural. With a new colleague or a client, expand unfamiliar terms and check understanding. Keep disagreement directed at the evidence or proposal, and match the degree of certainty to what is known.

Independent study

Speak each role aloud. Pause before the reply and supply your own response before revealing the next line. Record a second version using invented details, then compare its clarity and tone with the script.

Professional context

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

FULL DIALOGUE 01 / 7 SPEAKING TURNS

TPP Debate: Scientifically Interesting vs Clinically Meaningful

A discovery team wants to advance a compound, but the development team is uncertain about the patient and evidence logic.

Roles: Discovery lead / Clinical development / Development program specialist

Discovery lead: The biomarker response is strong. We should move quickly.

Clinical development: The biology is promising, but the clinical target is not clear yet.

Development program specialist: Before we commit to the next study, we need a sharper TPP: target population, expected clinical benefit, dose rationale, safety assumptions, comparator, endpoint, and differentiation from standard of care.

Discovery lead: The mechanism is novel.

Development program specialist: Novel mechanism helps, but it does not replace evidence that patients, regulators, physicians, and payers will consider meaningful.

Discovery lead: I will revise the target product profile around the development question.

Development program specialist: Keep the evidence gaps visible so the next study has a clear purpose.

Notice the professional language

TPP - Target product profile describing desired product attributes and evidence needs.

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 02 / 7 SPEAKING TURNS

IND Readiness: CMC Is Not Just a Detail

A team wants to file an IND quickly, but CMC documentation is incomplete.

Roles: Program lead / CMC lead / Development program specialist

Program lead: Clinical is ready. Let's file the IND next month.

CMC lead: The stability package and analytical validation are not complete.

Development program specialist: An IND is not only a protocol. We need enough nonclinical, clinical, and CMC information to support safe human dosing and product quality. If CMC is incomplete, the clinical timeline may not be credible.

Program lead: Can we explain that the data are coming?

Development program specialist: We can propose a plan, but we should not present future work as current readiness. Let's identify the minimum data needed and the regulatory risk of filing early.

Program lead: I will reconcile the CMC milestones with the submission plan.

Development program specialist: Then regulatory can assess readiness using the actual available package.

Notice the professional language

IND - Investigational new drug application allowing clinical investigation in humans in the United States.

CMC - Chemistry, manufacturing, and controls information supporting product quality.

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 03 / 7 SPEAKING TURNS

Endpoint Debate: Convenient vs Clinically Relevant

The team is choosing a primary endpoint for a Phase 2 trial.

Roles: Clinical operations / Medical lead / Development program specialist / Statistician

Clinical operations: The biomarker endpoint is easier to measure and faster.

Medical lead: But it may not persuade physicians.

Development program specialist: We should separate operational convenience from clinical relevance. If the biomarker is the primary endpoint, we need a strong rationale that it predicts patient benefit or supports the development decision.

Statistician: The study is not powered for the clinical endpoint.

Development program specialist: Then we should be explicit: the study may be decision-enabling for dose and signal, but not definitive for clinical benefit.

Clinical operations: I will document what this study can and cannot establish.

Development program specialist: Use that distinction consistently in the protocol rationale and readout.

Notice the professional language

Primary endpoint - Main outcome measure used to evaluate the primary objective.

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 04 / 7 SPEAKING TURNS

Enrollment Rescue: Protocol Burden and Site Reality

Enrollment is behind plan and the team is considering more sites.

Roles: Clinical operations / Site manager / Development program specialist / Program lead

Clinical operations: We are 40 percent behind enrollment. We should activate more sites.

Site manager: The current sites say the visit schedule is too burdensome.

Development program specialist: More sites may help, but only if the protocol is feasible. We should examine screen failure reasons, visit burden, competing trials, eligibility criteria, and patient travel before adding cost.

Program lead: We need speed.

Development program specialist: Agreed, but speed without feasibility will only create more underperforming sites. Let's propose a rescue plan with root cause, amendment options, and site-support actions.

Clinical operations: I will compile the screen-failure reasons and site feedback.

Development program specialist: Any proposed protocol change must follow the required review and approval process.

Notice the professional language

Enrollment rescue should start with root cause, not only site count.

Protocol burden language helps connect operations to data quality.

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 05 / 7 SPEAKING TURNS

Safety Signal: Possible Liver Risk

Several liver enzyme elevations have appeared across studies.

Roles: Safety physician / Clinical lead / Development program specialist / Regulatory affairs

Safety physician: We have a cluster of ALT elevations.

Clinical lead: Are they related to the drug?

Development program specialist: We should not jump to causality, but we also should not minimize it. Let's review timing, dose relationship, dechallenge, rechallenge, baseline risk factors, concomitant medications, seriousness, and whether the pattern changes the risk-benefit assessment.

Regulatory affairs: Do we need to notify FDA?

Development program specialist: Let's assess seriousness, expectedness, reasonable possibility, and reporting timelines against the protocol, regulations, and safety SOPs.

Safety physician: I will route the case information promptly through pharmacovigilance.

Development program specialist: Record the evidence and follow-up without implying causality has already been established.

Notice the professional language

Safety language should be calm, structured, and urgent where needed.

Avoid both premature causality and premature reassurance.

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 06 / 7 SPEAKING TURNS

Data Readout: Statistically Significant but Modest Effect

A Phase 3 trial met the primary endpoint, but the effect size is small and safety events increased.

Roles: Executive / Biostatistician / Development program specialist / Commercial

Executive: The primary endpoint met the prespecified statistical criterion. Can we call this a major breakthrough?

Biostatistician: The effect is statistically significant, but the confidence interval is narrow around a modest benefit.

Development program specialist: We can say the primary endpoint was met, but we should not overstate clinical impact. The communication needs effect size, clinical relevance, safety profile, population, and comparison to current options.

Commercial: But we need a strong launch message.

Development program specialist: A strong message must still be supportable. Overclaiming now may create regulatory, credibility, and MLR risk later.

Executive: I will revise the claim to reflect the effect size and uncertainty.

Development program specialist: Submit the complete context for the required medical, legal, and regulatory review.

Notice the professional language

Confidence interval - An interval produced by a method with a stated long-run coverage rate under its assumptions.

Primary endpoint - Main outcome measure used to evaluate the primary objective.

MLR - Medical, legal, and regulatory review of materials.

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 07 / 7 SPEAKING TURNS

Quality Deviation: Batch Release Pressure

A batch is needed for launch supply, but an OOS result is under investigation.

Roles: Supply lead / Quality / Development program specialist

Supply lead: If we do not release this batch, launch inventory is at risk.

Quality: The OOS investigation is not closed.

Development program specialist: The open investigation needs the required quality assessment and authorized disposition. We should not promise release while that process remains unresolved; supply planning needs an accurate statement of the current status.

Supply lead: Can we release with a memo?

Development program specialist: A memo is not a substitute for the applicable quality process. The authorized quality team must assess the evidence and determine the appropriate disposition under the current requirements.

Supply lead: I will communicate the pending batch status to supply planning.

Development program specialist: Quality will provide the authorized disposition through the controlled process.

Notice the professional language

OOS - Out of specification result requiring investigation.

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 08 / 7 SPEAKING TURNS

Promotional Claim Review: Subgroup Data

Marketing wants to use a subgroup result in launch materials.

Roles: Marketing / Medical reviewer / Development program specialist

Marketing: The subgroup response looks impressive. Can we make it the headline?

Medical reviewer: It was exploratory and not powered.

Development program specialist: The subgroup may be scientifically interesting, but as a promotional headline it could be misleading. We need to check whether the claim is consistent with approved labeling, adequately supported, and presented with appropriate context and risk information.

Marketing: Can we say 'especially effective' in that subgroup?

Development program specialist: Not unless that claim is supported and approved. We can discuss a balanced, label-consistent statement or keep the subgroup in scientific exchange if appropriate.

Marketing: I will remove the unsupported headline from the draft.

Development program specialist: Keep the exploratory analysis clearly identified in any material submitted for review.

Notice the professional language

Scientific exchange - Non-promotional scientific communication, usually handled within medical affairs boundaries.

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 09 / 7 SPEAKING TURNS

MSL Boundary: Unsolicited Off-Label Question

A physician asks an MSL about an off-label use during a scientific exchange.

Roles: Physician / MSL / Development program specialist

Physician: Have you seen data for use in younger patients?

MSL: That is outside the approved indication.

Development program specialist: I can route your unsolicited scientific question through our approved medical-information process. We need to distinguish the current approved indication from any other use and provide an appropriately reviewed, balanced response.

Physician: So you have data?

Development program specialist: I will confirm what information is available and arrange a response through that process, including study design, limitations, and the relevant context. I cannot promise a particular conclusion before reviewing the evidence.

Physician: I will record the question and preferred response channel.

Development program specialist: The designated medical-information team can then provide the appropriately reviewed response.

Notice the professional language

Boundary language should be clear without shutting down legitimate scientific exchange.

Balance and context matter in medical affairs conversations.

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 10 / 7 SPEAKING TURNS

Payer Evidence: Value Story Is Not the Same as Label

Market access is preparing for payer discussions after approval.

Roles: Market access / HEOR / Development program specialist / Commercial

Market access: Payers will ask why this deserves preferred formulary status.

HEOR: We have clinical data but limited real-world outcomes.

Development program specialist: The payer value story should connect eligible population, clinical benefit, safety, budget impact, comparator, adherence, and unmet need. We should not imply outcomes we have not measured.

Commercial: Can we say it reduces hospitalizations?

Development program specialist: Only if the evidence supports that claim for the relevant population and context. Otherwise, we can frame it as a hypothesis or evidence-generation need.

Market access: I will separate measured outcomes from modeled assumptions.

Development program specialist: That makes the evidence-generation needs clear without implying unobserved benefits.

Notice the professional language

Market access language must separate approved label, evidence, and economic hypothesis.

Payer communication should avoid unsupported outcomes claims.

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 11 / 7 SPEAKING TURNS

Biosimilar Discussion: Similar Is Not Identical

A cross-functional team is preparing educational material on a biosimilar.

Roles: Commercial / Regulatory affairs / Development program specialist / Medical affairs

Commercial: Can we say it is the same as the reference product?

Regulatory affairs: Biosimilar language is more specific than that.

Development program specialist: We should say highly similar with no clinically meaningful differences in safety, purity, and potency, consistent with the approved biosimilar framework. 'Same' may oversimplify and create confusion.

Medical affairs: What about interchangeability?

Development program specialist: Interchangeability has a specific regulatory meaning. We should use it only if the product has that designation and the material explains it accurately.

Commercial: I will verify the current product status and jurisdiction-specific terminology.

Development program specialist: Use the reviewed wording consistently in the final material.

Notice the professional language

Biosimilar language needs regulatory precision.

Avoid simplifying technical terms into misleading equivalence.

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 12 / 7 SPEAKING TURNS

Launch Readiness: Approval Is Not Enough

The team is two months from expected approval and launch readiness is uneven.

Roles: General manager / Launch lead / Development program specialist

General manager: If approval comes on time, we launch immediately.

Launch lead: Supply, MLR materials, medical training, payer coverage, and PV intake process still have open risks.

Development program specialist: Approval is necessary but not sufficient for launch. We need a readiness view across label, supply, quality release, safety reporting, medical information, field training, access, and promotional materials.

General manager: What is the critical path?

Development program specialist: Supply release, final label-dependent MLR approval, safety intake readiness, and payer communication are the main dependencies. I recommend a weekly risk review until launch decision.

General manager: I will assign owners to the open readiness items.

Development program specialist: Bring the evidence of completion to the launch review before requesting a decision.

Notice the professional language

Medical information - Function that responds to medical inquiries with approved, balanced, evidence-based information.

MLR - Medical, legal, and regulatory review of materials.

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

Eight more situations to make your own

The following cases are open role plays. They give you facts, contrasting roles, useful expressions, and a short model response. Use what you learned from the complete dialogues to create a longer exchange.

Preparation: two minutes. Conversation: three to five minutes. Feedback: one minute. Switch roles and repeat with the second-round challenge.

ROLE-PLAY CASE 01

Drug Development Strategy, Unmet Need, TPP, and Evidence Logic

SHARED CASE

A development slide describes an early laboratory result as evidence that a product will benefit patients. Clinical benefit has not been established.

Role A | Responding professional

Separate observations, assumptions, and conclusions. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You need to tell a colleague what is known. Ask which part is confirmed and which part is an assumption. Challenge one statement that sounds more certain than the case supports.

Useful expressions

The available evidence shows

This may indicate ..., but

We have not yet established whether

Cover this until after your first attempt

The result supports further investigation in this laboratory setting. It does not establish patient benefit. I'll revise the wording to distinguish the early finding from the clinical questions still to be tested.

Second round

Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 02

Regulatory Pathways, IND Readiness, NDA/BLA Strategy, and Agency Interaction

SHARED CASE

A team is preparing a question for a regulatory meeting. The draft asks broadly whether the whole development plan is acceptable.

Role A | Responding professional

Make a request with a clear action, purpose, and realistic timing. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You can help but need a clear request and its purpose. Ask what is required and whether the timing is fixed or negotiable.

Useful expressions

Could you provide ... so that ...?

Please confirm ... by [agreed time].

If that timing is not possible, please let me know

Cover this until after your first attempt

Could we identify the specific issue, proposed approach, and evidence supporting it? A focused question will make the requested feedback clearer than asking for general approval of the plan.

Second round

The other person cannot meet the requested timing. Clarify an alternative without inventing authority to change a formal deadline.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 03

Clinical Trial Design, Protocols, GCP, and Operations

SHARED CASE

Two trial documents give different visit-window descriptions. A coordinator is asked which one to use but has not confirmed the controlling version.

Role A | Responding professional

Clarify missing information before responding. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You need to know exactly what information is missing. Give one answer only after your partner asks a focused question; then ask them to confirm their understanding.

Useful expressions

When you say ..., do you mean ...?

Could you clarify which ...?

So, my understanding is Is that right?

Cover this until after your first attempt

The documents appear inconsistent. Which approved version governs this activity? I'll raise the discrepancy through the study process and request clarification before communicating a definitive instruction.

Second round

The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 04

Endpoints, Estimands, Statistics, Data Readouts, and Clinical Meaning

SHARED CASE

A trial slide highlights a secondary endpoint while the primary endpoint result needs explanation. The audience may mistake the highlighted result for the main study conclusion.

Role A | Responding professional

Separate observations, assumptions, and conclusions. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You need to tell a colleague what is known. Ask which part is confirmed and which part is an assumption. Challenge one statement that sounds more certain than the case supports.

Useful expressions

The available evidence shows

This may indicate ..., but

We have not yet established whether

Cover this until after your first attempt

Let's state the primary endpoint result clearly and identify the secondary analysis as such. The presentation should explain what each result supports and any limits on interpretation.

Second round

Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 05

Pharmacovigilance, Safety Signals, Risk-Benefit, and Label Updates

SHARED CASE

A team member receives a report of a possible adverse event. Key details are incomplete, and the team has a defined safety-reporting process.

Role A | Responding professional

Give a handoff that the receiving person can confirm. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are taking over the work. Ask what remains open and repeat back the critical status or responsibility. Point out one detail that would be unsafe or misleading to assume.

Useful expressions

The current status is ...; the outstanding item is

The record shows

Please confirm who has accepted

Cover this until after your first attempt

I've received a report that needs review through the safety process. Here are the details available and the information still missing. Please confirm receipt; I have not determined causality or classification.

Second round

The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 06

CMC, CGMP, Quality Events, Manufacturing, and Supply

SHARED CASE

A quality review identifies missing documentation in a batch record. A manager asks whether the batch can be described as fully reviewed.

Role A | Responding professional

Distinguish completed work from pending work and explain its impact. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You need to brief someone who has only one minute. Ask what is complete, what is open, and which open item affects the next action.

Useful expressions

We have completed ...; ... remains open.

As of ..., the status is

The next update will cover

Cover this until after your first attempt

The documentation review is still open. I'll report the missing items and their review status. We should not describe the batch review as complete until the responsible quality team confirms it.

Second round

The listener asks you to reduce the update to two sentences. Preserve the most important completed and pending items.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 07

Labeling, Medical Affairs, Promotion Review, and Compliance Boundaries

SHARED CASE

A draft promotional slide uses stronger benefit language than the approved material supplied for review.

Role A | Responding professional

State a concern respectfully and propose a practical route forward. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You want to move quickly and initially overlook a relevant condition. Ask why it matters, then consider a practical route forward that respects the real constraint.

Useful expressions

I understand the goal. My concern is

That statement goes beyond

Could we ... before confirming ...?

Cover this until after your first attempt

The proposed wording goes beyond the material we have been asked to use. Could we revise it to match the approved claim and send the change through the appropriate review process?

Second round

The other person repeats the request more firmly. Restate the specific concern calmly and explain the appropriate next route.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 08

Market Access, RWE, Launch Readiness, Biosimilars, and Lifecycle Management

SHARED CASE

A market-access presentation combines trial findings and observational data without identifying the different study designs.

Role A | Responding professional

Explain a useful distinction in plain English. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are unfamiliar with one technical term in the case. Ask for a plain-English explanation and then paraphrase it. Do not pretend to understand a term you cannot explain.

Useful expressions

In this context, ... means

The difference is that

How would you explain that distinction to a colleague?

Cover this until after your first attempt

Let's label the evidence sources separately. The trial and observational study answer different questions and have different limitations. That distinction will help the audience interpret the findings accurately.

Second round

Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

Lesson conversations and speaking practice

The following sections match 05 and 06 on the industry webpage and in the learner workbook. Each lesson has three ten-turn conversations and two bounded speaking scenarios. These are original fictional teaching scripts, not transcripts of real people.

LESSON 01 / 05 • CONVERSATIONS / 1 OF 3

A laboratory signal is not patient benefit

Fictional development-slide review: a laboratory result is promising, but no clinical benefit has been established.

01 • Development lead: This slide says our candidate will benefit patients. What evidence supports that wording?

02 • Research scientist: We observed the intended response in the laboratory system, not in patients.

03 • Development lead: Could we describe that as confirmation of the proposed mechanism of action, or MOA?

04 • Research scientist: That is still too strong. The result supports our hypothesis under these experimental conditions.

05 • Development lead: Then how should we connect it to the target product profile, the TPP?

06 • Research scientist: Present the TPP as a development target, with clinical benefit listed as unestablished.

07 • Development lead: Should the indication remain on the slide as the proposed patient population?

08 • Research scientist: Yes, provided we label it proposed and avoid implying an approved use.

09 • Development lead: I'll replace the benefit claim and separate evidence from the development objective.

10 • Research scientist: I'll check that the laboratory conditions and remaining evidence gaps stay visible.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 05 • CONVERSATIONS / 2 OF 3

Two indications, one evidence package

Separate fictional portfolio discussion: candidate P7 has laboratory data for indication A; indication B has only a biological rationale.

01 • Portfolio manager: Can we put indications A and B at the same development stage?

02 • Translational scientist: Not on this evidence. P7 has laboratory data for A and a rationale for B.

03 • Portfolio manager: The proposed MOA is similar. Doesn't that support the same conclusion for both?

04 • Translational scientist: It supports asking the question, not assuming that the results will transfer.

05 • Portfolio manager: How do we explain the difference without making B sound irrelevant?

06 • Translational scientist: Call B an exploratory opportunity and identify the experiment needed to assess it.

07 • Portfolio manager: Would a separate TPP make that distinction clearer for the committee?

08 • Translational scientist: Possibly, but its targets must not appear to be measured product characteristics.

09 • Portfolio manager: I'll show observed evidence for A beside the unanswered questions for B.

10 • Translational scientist: Good. The committee can discuss priorities without confusing potential with demonstrated performance.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 05 • CONVERSATIONS / 3 OF 3

Readiness for discussion versus readiness for filing

Separate fictional project check-in: an IND planning discussion is scheduled, but the supporting evidence inventory is incomplete.

01 • Project manager: The calendar says IND readiness. Can I report that the package is ready?

02 • Regulatory scientist: No. That is the meeting topic; our evidence inventory is still incomplete.

03 • Project manager: I understood the title to mean an investigational new drug submission was imminent.

04 • Regulatory scientist: We're reviewing gaps before proposing a filing schedule, not confirming a submission date.

05 • Project manager: Which part of the update can I state with confidence today?

06 • Regulatory scientist: The planning discussion is scheduled. The package assessment and timing remain open.

07 • Project manager: Should I include the proposed indication and TPP in that discussion?

08 • Regulatory scientist: Yes. They explain what we intend to develop and why particular evidence matters.

09 • Project manager: I'll correct the milestone wording and avoid treating a meeting as a completed assessment.

10 • Regulatory scientist: I'll bring the inventory and clearly distinguish available documents from expected ones.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Drug Development Strategy, Unmet Need, TPP, and Evidence Logic

A development slide describes an early laboratory result as evidence that a product will benefit patients. Clinical benefit has not been established.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to tell a colleague what is known. Ask which part is confirmed and which part is an assumption. Challenge one statement that sounds more certain than the case supports.

Second round: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Model for scenario 1: The result supports further investigation in this laboratory setting. It does not establish patient benefit. I'll revise the wording to distinguish the early finding from the clinical questions still to be tested.

Scenario 2 | An unmet-need claim without a source

In a fictional portfolio meeting, a slide says all existing options fail patients. The team has interviewed six clinicians, but no comparative evidence review is available. Discuss wording that preserves the development rationale without claiming universal failure.

Role A: Development strategist: explain what the interviews support and propose a qualified unmet-need statement.

Role B: Medical reviewer: challenge the universal wording and ask which evidence is still needed.

Possible opening: The six interviews identify concerns, but they don't establish that every existing option fails.

Success checks

- Distinguish reported concerns from a general clinical conclusion.
- Connect the proposed indication or TPP to an unanswered evidence question.
- Agree on a spoken claim that retains the limitation.

Add a complication: The presenter wants to keep the absolute claim because the speaking slot is only one minute.

LESSON 02 / 05 • CONVERSATIONS / 1 OF 3

A regulatory question the team can actually discuss

Fictional preparation for an agency meeting: the draft question asks whether the entire development plan is acceptable.

01 • Regulatory lead: Our question asks whether the whole plan is acceptable. What specific feedback do we need?

02 • Clinical lead: We need feedback on the proposed evidence supporting the next development stage.

03 • Regulatory lead: That still covers several topics. Which uncertainty affects our immediate planning decision?

04 • Clinical lead: Whether our proposed study population addresses the question described in the briefing document.

05 • Regulatory lead: Then let's ask about that population and provide our rationale, rather than request blanket approval.

06 • Clinical lead: Should we mention the IND in the opening sentence for context?

07 • Regulatory lead: Yes, but distinguish our current planning question from any future submission outcome.

08 • Clinical lead: I'll revise the question and identify the relevant briefing section this afternoon.

09 • Regulatory lead: Please send it for internal review before we finalize the meeting materials.

10 • Clinical lead: Understood. I'll request focused feedback without suggesting that a discussion guarantees acceptance.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 05 • CONVERSATIONS / 2 OF 3

Confirming which submission label belongs in the plan

Separate fictional planning meeting: a slide lists NDA/BLA as interchangeable placeholders; the regulatory strategy has not been confirmed.

01 • Program manager: Can you select NDA or BLA so our timeline looks complete?

02 • Regulatory strategist: I can review the terminology, but I shouldn't choose a route for appearance.

03 • Program manager: What information do you need from the technical team to assess it?

04 • Regulatory strategist: Please provide the current product description and the assumptions behind the proposed submission strategy.

05 • Program manager: Could we call both labels equivalent until that review is finished?

06 • Regulatory strategist: No. A new drug application and a biologics license application are distinct submission terms.

07 • Program manager: I'll mark the route as under review. Can you assess the description before Thursday's planning session?

08 • Regulatory strategist: I can give an initial assessment then, with unresolved questions clearly identified.

09 • Program manager: That will let us discuss dependencies without presenting an invented regulatory conclusion.

10 • Regulatory strategist: Agreed. I'll also specify what further specialist review is needed before the label is finalized.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 05 • CONVERSATIONS / 3 OF 3

Assigning questions after a response letter

Separate fictional internal exercise: a complete response letter lists three issues, and the team has not yet assigned reviewers.

01 • Regulatory coordinator: We have three issues in the complete response letter and no assigned reviewers yet.

02 • Program lead: Can we tell the board that resolving those issues guarantees approval?

03 • Regulatory coordinator: No. We can describe our response planning, not promise the outcome of another review.

04 • Program lead: What action do you need from me before tomorrow's internal meeting?

05 • Regulatory coordinator: Please nominate an accountable reviewer for each issue and confirm their availability.

06 • Program lead: Should those reviewers commit to completion dates immediately, before assessing the requested work?

07 • Regulatory coordinator: Ask for initial scope estimates first. Some dependencies may still be unknown.

08 • Program lead: I'll request owners today and ask them to identify unanswered questions tomorrow.

09 • Regulatory coordinator: I'll keep each response item linked to the relevant wording in the letter.

10 • Program lead: Then our update can distinguish assigned work, estimated timing, and decisions we cannot predict.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Regulatory Pathways, IND Readiness, NDA/BLA Strategy, and Agency Interaction

A team is preparing a question for a regulatory meeting. The draft asks broadly whether the whole development plan is acceptable.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You can help but need a clear request and its purpose. Ask what is required and whether the timing is fixed or negotiable.

Second round: The other person cannot meet the requested timing. Clarify an alternative without inventing authority to change a formal deadline.

Model for scenario 1: Could we identify the specific issue, proposed approach, and evidence supporting it? A focused question will make the requested feedback clearer than asking for general approval of the plan.

Scenario 2 | An unanswered meeting-minutes discrepancy

In a fictional regulatory follow-up, internal notes say a proposed analysis was discussed; the draft summary says it was agreed. No final agency minutes are available. Conduct a call to request a precise wording review before circulation tomorrow.

Role A: Regulatory coordinator: identify the discrepancy and request review of the specific sentence.

Role B: Clinical representative: explain the internal recollection without claiming confirmed agreement.

Possible opening: Our notes record discussion, but the summary says agreement; could you review that distinction today?

Success checks

- Identify the exact wording needing review.
- Distinguish internal recollection from confirmed agency feedback.
- Agree an owner and a realistic response time without inventing approval.

Add a complication: A project sponsor asks you to circulate the stronger version immediately.

LESSON 03 / 05 • CONVERSATIONS / 1 OF 3

Which visit window controls?

Fictional trial coordination call: two documents describe different visit windows, and the controlling version is unconfirmed.

01 • Site coordinator: The visit guide and protocol give different windows. Which one should I communicate?

02 • Clinical monitor: Before answering, could you confirm the version identifiers on both documents?

03 • Site coordinator: I can read them back, but I haven't checked which version is controlling.

04 • Clinical monitor: Then let's verify that through document control instead of choosing the more convenient window.

05 • Site coordinator: The scheduling team wants an answer now. Can I call the difference a protocol deviation?

06 • Clinical monitor: Not from the discrepancy alone. First establish the applicable requirement and what actually happened.

07 • Site coordinator: I'll explain that the scheduling instruction is awaiting confirmation, without giving a replacement window.

08 • Clinical monitor: Please send the conflicting passages through the trial's authorized document channel.

09 • Site coordinator: Will you take ownership of checking the controlling version and returning a clear answer?

10 • Clinical monitor: Yes. I'll confirm the requirement with the responsible team and close the communication loop.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 05 • CONVERSATIONS / 2 OF 3

A consent discussion is not a completed consent record

Separate fictional training case: a tracker says consent discussed, while a meeting agenda says ready for randomization.

01 • Trial coordinator: Our agenda says ready for randomization, but the tracker only says consent discussed.

02 • Study nurse: That wording doesn't tell us whether the required informed consent process is complete.

03 • Trial coordinator: Could the two entries simply mean the same thing in different words?

04 • Study nurse: We need confirmation, not an assumption. Which authorized record supports the readiness status?

05 • Trial coordinator: I haven't checked it, and I don't have a verified completion entry.

06 • Study nurse: Then ask the responsible site clinician to confirm the status under the protocol.

07 • Trial coordinator: Should I change the agenda to consent status awaiting verification for now?

08 • Study nurse: Yes. That describes our knowledge without implying either completion or a confirmed failure.

09 • Trial coordinator: I'll also ask who updates the tracker once the status is verified.

10 • Study nurse: Good. We'll use the confirmed record for the next communication, not infer readiness from a heading.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 05 • CONVERSATIONS / 3 OF 3

Allocation wording in a recruitment briefing

Separate fictional recruitment rehearsal: a draft describes randomization as the coordinator choosing the most suitable study group.

01 • Recruitment coordinator: The briefing says I'll choose the most suitable group. Is that our randomization process?

02 • Trial educator: That wording sounds like personal selection. What does the approved protocol explanation say?

03 • Recruitment coordinator: It describes assignment through the study's randomization process, without a coordinator choosing.

04 • Trial educator: Then we should use that explanation and remove the suggestion of individual selection.

05 • Recruitment coordinator: Could I explain the exact allocation ratio to make the process clearer?

06 • Trial educator: Only after checking the approved information. We haven't established that ratio in this rehearsal.

07 • Recruitment coordinator: I'll ask which participant-facing wording is approved before the next practice session.

08 • Trial educator: Also distinguish explaining assignment from making promises about which group someone will enter.

09 • Recruitment coordinator: Understood. I'll clarify the process and refer questions beyond the materials to the study team.

10 • Trial educator: Let's rehearse that corrected explanation once the source wording has been confirmed.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Clinical Trial Design, Protocols, GCP, and Operations

Two trial documents give different visit-window descriptions. A coordinator is asked which one to use but has not confirmed the controlling version.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to know exactly what information is missing. Give one answer only after your partner asks a focused question; then ask them to confirm their understanding.

Second round: The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

Model for scenario 1: The documents appear inconsistent. Which approved version governs this activity? I'll raise the discrepancy through the study process and request clarification before communicating a definitive instruction.

Scenario 2 | Remote or on-site visit?

A fictional trial calendar labels a follow-up remote, while a training checklist says on-site. The participant has not been contacted, and the applicable protocol version is unknown. Discuss how to obtain a reliable scheduling instruction.

Role A: Site scheduler: ask focused questions about the visit type and controlling document.

Role B: Trial operations specialist: acknowledge what is unknown and take responsibility for verification.

Possible opening: Before I contact the participant, could we confirm whether this visit is remote or on-site?

Success checks

- Do not treat the most recent edit as proof of authority.
- Name the document and visit details requiring confirmation.
- Agree who returns the verified instruction before participant contact.

Add a complication: A colleague suggests using the calendar because it was edited most recently.

LESSON 04 / 05 • CONVERSATIONS / 1 OF 3

The highlighted endpoint is not the primary result

Fictional trial-slide review: a secondary endpoint is highlighted, while the primary endpoint result needs explanation.

01 • Medical writer: The headline highlights the secondary endpoint. Will listeners think that was the main study question?

02 • Biostatistician: They may. We need the primary endpoint result and its interpretation visible first.

03 • Medical writer: Can I simply describe the highlighted result as a successful trial outcome?

04 • Biostatistician: That would exceed what this slide establishes. The endpoint hierarchy affects the interpretation.

05 • Medical writer: What should the audience hear before we discuss the additional result?

06 • Biostatistician: Which endpoint was primary, what was observed, and what uncertainty remains in that estimate.

07 • Medical writer: Should the confidence interval stay beside the estimate rather than in a footnote?

08 • Biostatistician: Yes. Keep uncertainty accessible, and have the full statistical interpretation reviewed.

09 • Medical writer: I'll reorganize the slide without inventing a conclusion about overall study success.

10 • Biostatistician: I'll check the endpoint descriptions and explain any limitations on the secondary analysis.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 05 • CONVERSATIONS / 2 OF 3

A precise number with an imprecise estimate

Separate fictional analysis rehearsal: an estimated difference is four units with a confidence interval from minus two to ten; clinical importance has not been assessed.

01 • Clinical scientist: The difference is four units. Can we call that a meaningful improvement?

02 • Biostatistician: Not from the point estimate alone. The confidence interval runs from minus two to ten.

03 • Clinical scientist: So should I say there is definitely no difference between the groups?

04 • Biostatistician: No. That also overstates the result. Explain the estimate and its uncertainty.

05 • Clinical scientist: What about calling four units clinically important for this outcome measure?

06 • Biostatistician: We haven't assessed clinical importance here. Don't supply a threshold we haven't established.

07 • Clinical scientist: I'll state the numbers, then separate statistical uncertainty from the clinical interpretation.

08 • Biostatistician: Please also identify the outcome measure and analysis population in the supporting material.

09 • Clinical scientist: Those details need verification before I present them, so I'll request the analysis source.

10 • Biostatistician: Good. We can give an accurate numerical summary without turning it into an unsupported benefit claim.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 05 • CONVERSATIONS / 3 OF 3

Two analyses answer different questions

Separate fictional methods discussion: two trial analyses handle treatment discontinuation differently; neither result is supplied in the exercise.

01 • Clinical lead: Why do we need two analysis descriptions if they concern the same treatment?

02 • Statistician: They handle treatment discontinuation differently, so their estimands may address different treatment-effect questions.

03 • Clinical lead: Is an estimand just another name for the statistical calculation we run?

04 • Statistician: No. It specifies the effect we want to estimate; the calculation is a separate matter.

05 • Clinical lead: Could we select whichever analysis eventually produces the more attractive estimate?

06 • Statistician: We should explain the prespecified question and analysis plan, not choose a story after seeing results.

07 • Clinical lead: What can I tell the medical team while the numerical results are unavailable?

08 • Statistician: Describe the differing questions and the handling of discontinuation, without claiming which result is stronger.

09 • Clinical lead: I'll ask you to review that explanation before the data-readout rehearsal.

10 • Statistician: Agreed. We'll make the interpretive boundary clear before discussing any eventual endpoint estimates.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Endpoints, Estimands, Statistics, Data Readouts, and Clinical Meaning

A trial slide highlights a secondary endpoint while the primary endpoint result needs explanation. The audience may mistake the highlighted result for the main study conclusion.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to tell a colleague what is known. Ask which part is confirmed and which part is an assumption. Challenge one statement that sounds more certain than the case supports.

Second round: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Model for scenario 1: Let's state the primary endpoint result clearly and identify the secondary analysis as such. The presentation should explain what each result supports and any limits on interpretation.

Scenario 2 | A subgroup headline without context

A fictional trial summary highlights one subgroup from an exploratory analysis. The overall primary endpoint result is available elsewhere but is not included in the summary. No interaction analysis is supplied. Discuss a responsible spoken introduction.

Role A: Medical communications lead: qualify the subgroup statement and request the missing context.

Role B: Study statistician: explain why the subgroup alone cannot establish a different treatment effect.

Possible opening: This is an exploratory subgroup finding, and we need the overall primary result beside it.

Success checks

- Identify the exploratory status.
- Avoid claiming a subgroup difference that has not been established.
- Agree which primary-result and uncertainty information must accompany the statement.

Add a complication: The presenter asks you to describe the subgroup as the patients who definitely benefit.

LESSON 05 / 05 • CONVERSATIONS / 1 OF 3

Passing an incomplete event report

Fictional safety-reporting exercise: a possible adverse event has been reported with incomplete details; the team has a defined reporting process.

01 • Medical information specialist: I received a possible adverse event report, but several details are missing.

02 • Safety associate: Please transfer the available information through our safety channel and identify what remains unknown.

03 • Medical information specialist: Should I wait until I can establish whether the product caused it?

04 • Safety associate: No. Our handoff should preserve the report without making a causality judgment.

05 • Medical information specialist: I also don't know whether the reported event meets the seriousness criteria.

06 • Safety associate: State that clearly. The responsible safety reviewer will assess the report under our process.

07 • Medical information specialist: I'll pass the reporter's wording and distinguish it from my own questions.

08 • Safety associate: Please include the available contact route in the restricted record, not general team chat.

09 • Medical information specialist: Can you confirm receipt and take responsibility for the next follow-up step?

10 • Safety associate: Yes. I'll acknowledge the handoff and flag the missing details for the assigned reviewer.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 05 • CONVERSATIONS / 2 OF 3

Severe wording does not settle seriousness

Separate fictional safety intake: a caller described an event as severe; hospitalization and other seriousness information are unknown.

01 • Intake coordinator: The caller said severe. I've prepared a handoff, but the seriousness details are unknown.

02 • Pharmacovigilance reviewer: Keep the caller's description. Don't silently change severe to serious in the record.

03 • Intake coordinator: I thought those words were interchangeable when describing an SAE, a serious adverse event.

04 • Pharmacovigilance reviewer: They address different concepts. We need the relevant case information and an authorized assessment.

05 • Intake coordinator: Should I classify it as non-serious because hospitalization wasn't mentioned during the call?

06 • Pharmacovigilance reviewer: No. Missing information is not evidence that a seriousness criterion is absent.

07 • Intake coordinator: I'll mark the details unknown and hand over the questions already asked.

08 • Pharmacovigilance reviewer: Thank you. I'll arrange follow-up through the case process and preserve the original wording.

09 • Intake coordinator: Please confirm that you now own follow-up, so the reporter doesn't receive duplicate calls.

10 • Pharmacovigilance reviewer: Confirmed. I'll coordinate the next contact and document the assessment when information is available.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 05 • CONVERSATIONS / 3 OF 3

A cluster passed to signal review

Separate fictional surveillance meeting: three reports describe a similar event, but duplicate status and exposure information have not been checked.

01 • Safety analyst: Three reports describe similar events. I'm handing them to you for signal review.

02 • Signal lead: Have you confirmed that they represent three separate cases rather than duplicate reports?

03 • Safety analyst: Not yet. Exposure information is also missing, so I haven't calculated an event rate.

04 • Signal lead: Good. Please preserve those limitations instead of presenting a confirmed increase in risk.

05 • Safety analyst: Can I say signal detection has established a causal association with the product?

06 • Signal lead: No. These reports prompt assessment; they don't establish causality or a risk-benefit conclusion.

07 • Safety analyst: I'll link the case references and summarize the shared wording without adding a diagnosis.

08 • Signal lead: I'll assign duplicate review and confirm what information is needed for further assessment.

09 • Safety analyst: Who should receive questions if another similar report arrives during your review?

10 • Signal lead: Route it through the existing safety process and reference this review so we can reconcile the records.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Pharmacovigilance, Safety Signals, Risk-Benefit, and Label Updates

A team member receives a report of a possible adverse event. Key details are incomplete, and the team has a defined safety-reporting process.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are taking over the work. Ask what remains open and repeat back the critical status or responsibility. Point out one detail that would be unsafe or misleading to assume.

Second round: The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

Model for scenario 1: I've received a report that needs review through the safety process. Here are the details available and the information still missing. Please confirm receipt; I have not determined causality or classification.

Scenario 2 | A reporter corrects the event date

A fictional safety case is already assigned to reviewer Mina. The reporter now says the recorded event date was a transcription error but cannot recall the correct date. Practice handing over this correction without deleting the history or inventing a replacement.

Role A: Medical information specialist: transfer the correction and identify the remaining uncertainty.

Role B: Safety reviewer Mina: confirm receipt and explain who coordinates further clarification.

Possible opening: The reporter has withdrawn the recorded date, but a replacement date is still unknown.

Success checks

- Separate the contact date from the unknown event date.
- Preserve the original report and correction through the case process.
- Confirm the receiving owner and next follow-up responsibility.

Add a complication: A colleague suggests using the first contact date as the event date to avoid an empty field.

LESSON 06 / 05 • CONVERSATIONS / 1 OF 3

A batch review with missing documentation

Fictional quality meeting: documentation is missing from a batch record, and a manager asks whether review is complete.

01 • Production manager: Can I describe the batch as fully reviewed in today's supply update?

02 • Quality reviewer: Not yet. The batch record review identified missing documentation that remains unresolved.

03 • Production manager: The manufacturing steps are complete. Isn't fully reviewed a reasonable shorthand for that?

04 • Quality reviewer: Those are different statuses. Completed production doesn't establish completed quality review or authorized disposition.

05 • Production manager: What wording will explain the situation without implying a confirmed product defect?

06 • Quality reviewer: Say production is complete and document review remains open because supporting information is missing.

07 • Production manager: Should I call this OOS in the update to emphasize its importance?

08 • Quality reviewer: No. An out-of-specification result is not established by the documentation gap described here.

09 • Production manager: I'll keep the supply forecast conditional and request the next review-status checkpoint.

10 • Quality reviewer: I'll provide that checkpoint and identify what remains outstanding without predicting the disposition decision.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 05 • CONVERSATIONS / 2 OF 3

An OOS investigation is not a release decision

Separate fictional quality exercise: one test result is recorded as OOS, the investigation is open, and no disposition is authorized.

01 • Supply planner: The investigation has started. Does that mean the batch will be available this week?

02 • Quality investigator: No. One result is out of specification, and the investigation and disposition remain open.

03 • Supply planner: Can I remove the delay from the supply dashboard while you repeat the test?

04 • Quality investigator: We haven't agreed a repeat test here. Don't assume a particular investigation step or outcome.

05 • Supply planner: What completed action can I report to the commercial planning team today?

06 • Quality investigator: The OOS result has been logged and assigned for investigation. Availability is not confirmed.

07 • Supply planner: Would describing it as a CMC quality issue help explain the dependency?

08 • Quality investigator: Yes, with the specific status. Broad terminology shouldn't hide the unresolved batch decision.

09 • Supply planner: I'll show an unconfirmed supply date and keep any alternative plan separate.

10 • Quality investigator: I'll update you when the authorized review changes status, not merely when an activity is scheduled.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 05 • CONVERSATIONS / 3 OF 3

CAPA implementation versus demonstrated effectiveness

Separate fictional CAPA review: a revised batch-record checklist is in use, but the planned effectiveness check has not occurred.

01 • Quality manager: The new checklist is in use. Can we close the corrective and preventive action?

02 • CAPA owner: Implementation is complete, but the planned effectiveness check hasn't happened yet.

03 • Quality manager: Staff say the checklist is clearer. Doesn't that demonstrate that the issue is resolved?

04 • CAPA owner: It is useful feedback, but it doesn't replace the agreed evidence for effectiveness.

05 • Quality manager: How should the progress report distinguish those two stages of the work?

06 • CAPA owner: List the revised checklist as implemented and the effectiveness assessment as pending.

07 • Quality manager: Could we describe the batch-record problem as eliminated across every production line?

08 • CAPA owner: Not on this evidence. Our change and planned assessment have a defined scope.

09 • Quality manager: I'll keep that scope visible and ask when the assessment results can be reviewed.

10 • CAPA owner: I'll confirm the assessment schedule and report findings without treating completion of training as proof of prevention.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | CMC, CGMP, Quality Events, Manufacturing, and Supply

A quality review identifies missing documentation in a batch record. A manager asks whether the batch can be described as fully reviewed.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to brief someone who has only one minute. Ask what is complete, what is open, and which open item affects the next action.

Second round: The listener asks you to reduce the update to two sentences. Preserve the most important completed and pending items.

Model for scenario 1: The documentation review is still open. I'll report the missing items and their review status. We should not describe the batch review as complete until the responsible quality team confirms it.

Scenario 2 | A supplier certificate arrives unread

In a fictional CMC supply meeting, the missing supplier certificate has arrived for lot L8. Quality has not reviewed it, and disposition remains pending. Discuss what changed and what did not change in the lot's status.

Role A: Materials coordinator: report receipt accurately without implying release.

Role B: Quality reviewer: clarify the remaining document check and take responsibility for the next update.

Possible opening: The certificate for L8 has arrived, but receipt is not the same as completed quality review.

Success checks

- Distinguish document receipt from review and disposition.
- Avoid assuming the certificate is acceptable before assessment.
- Agree who communicates the next verified status.

Add a complication: A planner asks to change the availability indicator to confirmed immediately.

LESSON 07 / 05 • CONVERSATIONS / 1 OF 3

A stronger adjective than the reviewed material

Fictional promotional review: a draft slide makes a stronger benefit claim than the supplied approved material.

01 • Brand manager: The stronger headline is clearer. Can we keep it for tomorrow's presentation?

02 • Medical reviewer: My concern is that it goes beyond the benefit language in the supplied approved material.

03 • Brand manager: Could a reference to the prescribing information at the bottom solve that problem?

04 • Medical reviewer: A reference doesn't make an unsupported headline accurate. We need the claim itself reviewed.

05 • Brand manager: What can we do within the remaining preparation time without inventing new wording?

06 • Medical reviewer: Use the supplied language as the starting point and route the revision through MLR.

07 • Brand manager: By MLR, you mean medical, legal, and regulatory review, not just your personal approval?

08 • Medical reviewer: Correct. Also preserve the relevant risk context and fair balance for the proposed material.

09 • Brand manager: I'll replace the stronger claim and keep the presentation status pending that review.

10 • Medical reviewer: Thank you. I'll identify the precise wording concern so the reviewers can address it efficiently.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 05 • CONVERSATIONS / 2 OF 3

Scientific exchange is not a label for any slide

Separate fictional conference preparation: a commercial deck contains unreviewed exploratory findings; its owner proposes calling the deck scientific exchange.

01 • Commercial lead: Could we call this scientific exchange and keep the exploratory findings in the deck?

02 • Medical affairs lead: Changing the label doesn't settle the purpose, audience, or appropriate review of the content.

03 • Commercial lead: The findings are interesting. Why can't the presenter simply describe them as promising?

04 • Medical affairs lead: We need to assess the actual claims and limitations, not rely on a reassuring adjective.

05 • Commercial lead: What practical alternative preserves the audience's opportunity to ask scientific questions?

06 • Medical affairs lead: Keep this presentation within its reviewed scope and refer scientific inquiries through the appropriate medical channel.

07 • Commercial lead: Should I send you the question and the study source separately from the deck?

08 • Medical affairs lead: Yes. We'll assess an appropriate response without assuming this commercial material is the right vehicle.

09 • Commercial lead: I'll remove the unreviewed section from the proposed presentation and flag the inquiry.

10 • Medical affairs lead: I'll coordinate the relevant review and avoid promising that any particular content will be approved.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 05 • CONVERSATIONS / 3 OF 3

A risk panel lost in a smaller format

Separate fictional design review: resizing a promotional panel has made the supplied risk text unreadable; no replacement layout is approved.

01 • Design lead: The compact version fits the screen, but the risk panel is now very small.

02 • Compliance reviewer: That creates a fair-balance concern. The benefit headline is prominent while the risk text is unreadable.

03 • Design lead: Could we keep the headline and move all risk context to a later slide?

04 • Compliance reviewer: I can't approve that shortcut. The revised presentation needs review in its actual format.

05 • Design lead: What change can I prepare that gives the reviewers something workable to assess?

06 • Compliance reviewer: Try a larger layout using the supplied content, with both benefit and risk information legible.

07 • Design lead: Would shortening the prescribing-information extract require a fresh content review as well?

08 • Compliance reviewer: Yes, route any proposed wording change through the relevant review rather than editing it independently.

09 • Design lead: I'll prepare a readable version and mark it as a draft for MLR.

10 • Compliance reviewer: Good. We'll review the complete presentation, including how the information appears to the audience.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Labeling, Medical Affairs, Promotion Review, and Compliance Boundaries

A draft promotional slide uses stronger benefit language than the approved material supplied for review.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You want to move quickly and initially overlook a relevant condition. Ask why it matters, then consider a practical route forward that respects the real constraint.

Second round: The other person repeats the request more firmly. Restate the specific concern calmly and explain the appropriate next route.

Model for scenario 1: The proposed wording goes beyond the material we have been asked to use. Could we revise it to match the approved claim and send the change through the appropriate review process?

Scenario 2 | A testimonial asks for a stronger promise

A fictional promotional video draft includes a speaker saying the product always works. The supplied approved material contains no universal benefit claim. Discuss a revision request before recording; no replacement statement has been reviewed.

Role A: Production coordinator: explain the recording constraint and ask for usable reviewed wording.

Role B: Medical reviewer: challenge the absolute claim and propose the review route.

Possible opening: The word always promises more than our supplied material supports, so this line needs review before recording.

Success checks

- Identify the specific unsupported promise.
- Avoid treating a testimonial label as an automatic exemption.
- Agree to obtain reviewed wording without inventing a new benefit claim.

Add a complication: The speaker says the sentence is a personal opinion and therefore needs no review.

LESSON 08 / 05 • CONVERSATIONS / 1 OF 3

Trial findings beside observational evidence

Fictional market-access rehearsal: trial findings and observational data appear together without study-design labels.

01 • Market access manager: The presentation combines these results. Why do we need separate study-design labels?

02 • HEOR scientist: Trial findings and observational analyses have different conditions and limitations.

03 • Market access manager: Does that mean the observational information has no place here?

04 • HEOR scientist: No. Health economics and outcomes research can use it, with its source and limits explained.

05 • Market access manager: How do I distinguish real-world data, RWD, from real-world evidence, RWE?

06 • HEOR scientist: RWD are routinely collected healthcare and patient data. RWE is clinical evidence about use, benefits, or risks derived from analyzing those data.

07 • Market access manager: Can I present both sources as equally establishing that the product caused the outcome?

08 • HEOR scientist: Not without examining the designs. Putting results together doesn't make their causal interpretations equivalent.

09 • Market access manager: I'll label each source and separate findings from any formulary recommendation.

10 • HEOR scientist: I'll check that our explanation of the limitations matches the actual analyses.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 05 • CONVERSATIONS / 2 OF 3

A formulary decision is not a universal access promise

Separate fictional launch meeting: one organization's formulary listing is confirmed, while patient-level access conditions have not been reviewed.

01 • Launch coordinator: We have a formulary listing. Can the field update say access is now unrestricted?

02 • Access specialist: No. The listing is confirmed, but we haven't reviewed the patient-level access conditions.

03 • Launch coordinator: I thought formulary meant everyone covered by that organization could obtain the product immediately.

04 • Access specialist: It identifies inclusion on a medicine list; the actual terms need separate checking.

05 • Launch coordinator: What distinction should the update make for colleagues planning launch activities this week?

06 • Access specialist: State the confirmed listing and flag that operational access details remain under review.

07 • Launch coordinator: Could HEOR results tell us those administrative conditions without obtaining the listing documents?

08 • Access specialist: No. Outcomes evidence and access requirements are different information sources for different questions.

09 • Launch coordinator: I'll request the relevant documents and remove the unrestricted-access phrase from the draft.

10 • Access specialist: I'll check the conditions and provide a scoped explanation rather than a promise about individual coverage.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 05 • CONVERSATIONS / 3 OF 3

Database size versus a usable comparison

Separate fictional RWE planning discussion: access to a database of 50,000 routinely collected patient records is confirmed, but completeness of the proposed outcome and comparator variables is unknown.

01 • Business development lead: Fifty thousand records sounds persuasive. Can we say the evidence package is already strong?

02 • RWE analyst: We have a large data source, but outcome and comparator completeness are still unknown.

03 • Business development lead: Does the number of records compensate for those missing details in the analysis?

04 • RWE analyst: Not automatically. Record count doesn't tell us whether the data can answer the proposed question.

05 • Business development lead: What should our lifecycle planning discussion distinguish before we promise a deliverable?

06 • RWE analyst: Data availability from analytical fitness. We need to assess the variables, population, and proposed comparison.

07 • Business development lead: Could we call this RWD access secured, with the RWE feasibility assessment pending?

08 • RWE analyst: Yes. Access is confirmed, but we shouldn't imply the analysis has already produced clinical evidence.

09 • Business development lead: I'll report the confirmed access separately from the pending assessment so the distinction stays clear.

10 • RWE analyst: I'll report feasibility findings before recommending an outcome claim or a timetable for the evidence package.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Market Access, RWE, Launch Readiness, Biosimilars, and Lifecycle Management

A market-access presentation combines trial findings and observational data without identifying the different study designs.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are unfamiliar with one technical term in the case. Ask for a plain-English explanation and then paraphrase it. Do not pretend to understand a term you cannot explain.

Second round: Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

Model for scenario 1: Let's label the evidence sources separately. The trial and observational study answer different questions and have different limitations. That distinction will help the audience interpret the findings accurately.

Scenario 2 | Launch activity versus access outcome

A fictional launch dashboard records twelve payer meetings but no formulary decisions yet. A slide calls the meetings twelve access wins. Discuss how to explain the activity and the unconfirmed outcome in a short leadership conversation.

Role A: Launch operations lead: explain what the twelve meetings measure.

Role B: Market-access director: ask which evidence would support a confirmed access outcome.

Possible opening: We completed twelve payer meetings; that is an activity count, not twelve confirmed formulary decisions.

Success checks

- Separate completed meetings from formulary outcomes.
- Do not invent payer decisions or coverage conditions.
- Agree a concise label and an owner for tracking decisions.

Add a complication: A colleague wants to keep wins because the dashboard has no space for caveats.

Give feedback that helps

Score each criterion 0, 1, or 2. This is a practice rubric, not a professional qualification or CEFR test. Do not award or remove points for a particular accent.

Meaning and accuracy

Keeps the case facts accurate; clearly separates confirmed and unknown information.

Clarity and language

Uses understandable sentences and explains specialist terms when the listener needs it.

Interaction and tone

Responds to the other person's question, checks understanding, and uses an appropriate tone.

Task completion

Makes the requested action or unresolved question clear without inventing authority or facts.

Use the same scale for each criterion

0 - Not yet: The reader or listener cannot yet recover the intended meaning or action.

1 - With support: The message works after a prompt, clarification, or revision.

2 - Independently: The message is clear and accurate without a prompt.

Feedback sequence

First, identify a successful phrase. Next, identify one point where meaning or accuracy needs work. Offer one useful language correction, allow a repeat, and compare the two attempts. A total out of eight describes this performance only; do not translate it into a proficiency level.

Final challenge

Choose a case not rehearsed today. The learner gives a one-minute response, answers two follow-up questions, and writes a 70-110 word message. Add the lesson's second-round challenge. Compare the result with an earlier attempt using the four criteria.

Use the language responsibly

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.

Extend your practice with AI

Use the following prompts after your own first attempt. Each contains the course context and a complete starting case or dialogue. Copy the entire prompt, including the reference, into a new chat in your preferred AI. You can also use the links to copy it from the website.

Choose the right kind of help

A role-play prompt makes the AI wait for your replies. A new-dialogue prompt asks for a complete professional script. Vocabulary and grammar prompts move from explanation to your own attempts.

Choose any lesson or dialogue online

The course prompt workshop has four practice goals and B1 support, B2 independent and C1 stretch settings. These are practice choices, not assessed proficiency levels. Select the same lesson number or dialogue title you used in this guide.

[Open this course's AI prompt workshop](#)

Keep practice useful

Use fictional or anonymized details. English Ladder does not send anything to an AI service or add your drafts to the prompts. Your chosen service's terms and any usage charges apply. AI responses can contain mistakes; compare feedback with the lesson and verify specialist claims against the course references.

Each starter prompt uses B2 practice. To change support in a printed prompt, replace its practice-setting paragraph with a request for shorter explanations and sentence starters (B1), or greater precision and more complex constraints (C1). Keep the professional facts unchanged.

Changing the example

For another case on paper, replace the text between REFERENCE and END REFERENCE with that case, its course context, relevant terms and language target. Include the full exchange if practicing a dialogue. Do not assume your AI can access this PDF or a link. The website makes this substitution for you.

Prompt edition: 2026-09-30. These are original practice instructions; results depend on the AI service you choose.

Rehearse with an AI colleague

START PROMPT / COPY THROUGH END PROMPT

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

REFERENCE

Course: Pharmaceutical English

Professional audience: clinical development, regulatory affairs, pharmacovigilance, quality, CMC, manufacturing, medical affairs, market access, and compliance teams

Scope: Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Dialogue 01: TPP Debate: Scientifically Interesting vs Clinically Meaningful

Original fictional setting: A discovery team wants to advance a compound, but the development team is uncertain about the patient and evidence logic.

Professional roles: Discovery lead / Clinical development / Development program specialist

Published dialogue (reference script, not my own performance):

Discovery lead: The biomarker response is strong. We should move quickly.

Clinical development: The biology is promising, but the clinical target is not clear yet.

Development program specialist: Before we commit to the next study, we need a sharper TPP: target population, expected clinical benefit, dose rationale, safety assumptions, comparator, endpoint, and differentiation from standard of care.

Discovery lead: The mechanism is novel.

Development program specialist: Novel mechanism helps, but it does not replace evidence that patients, regulators, physicians, and payers will consider meaningful.

Discovery lead: I will revise the target product profile around the development question.

Development program specialist: Keep the evidence gaps visible so the next study has a clear purpose.

Vocabulary in this exchange: TPP: Target product profile describing desired product attributes and evidence needs.

END REFERENCE

TASK: Interactive workplace role-play

1. Use the supplied case or dialogue as the starting situation. Give a two-sentence briefing and offer two relevant professional roles for me to choose from. For a supplied dialogue, use its actual roles. Ask which role I want, then stop and wait. Do not write my replies.
2. After I choose, identify who you will play and the immediate communication goal. Play the other professional; where a meeting requires a third person, label each of your speakers clearly. Start with one natural workplace turn and wait for my reply. Keep most turns to one to three sentences and ask no more than one question at a time.
3. Let the exchange develop across six to ten learner turns, or end earlier if I type "feedback". Respond to what I actually say. Include a plausible clarification, disagreement or tradeoff without silently changing the starting facts. Mark any added constraint as a fictional second-round variation. Do not resolve approvals, evidence or commitments that remain uncertain.
4. Stay in role during the exchange. If my meaning is unclear, ask for clarification naturally. Give a hint only if I ask or cannot proceed. Do not deliver a model conversation in advance.
5. At the debrief, quote two of my phrases: one successful choice and one worth improving. Check factual accuracy, language, register and whether the next step was clear. Give up to three focused suggestions. Ask me to retry the weakest turn; wait. Only then offer an alternative wording and a harder replay.

END PROMPT

Copy this prompt online or choose another lesson and support level

Create more scenario dialogues

START PROMPT / COPY THROUGH END PROMPT

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

REFERENCE

Course: Pharmaceutical English

Professional audience: clinical development, regulatory affairs, pharmacovigilance, quality, CMC, manufacturing, medical affairs, market access, and compliance teams

Scope: Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Dialogue 01: TPP Debate: Scientifically Interesting vs Clinically Meaningful

Original fictional setting: A discovery team wants to advance a compound, but the development team is uncertain about the patient and evidence logic.

Professional roles: Discovery lead / Clinical development / Development program specialist

Published dialogue (reference script, not my own performance):

Discovery lead: The biomarker response is strong. We should move quickly.

Clinical development: The biology is promising, but the clinical target is not clear yet.

Development program specialist: Before we commit to the next study, we need a sharper TPP: target population, expected clinical benefit, dose rationale, safety assumptions, comparator, endpoint, and differentiation from standard of care.

Discovery lead: The mechanism is novel.

Development program specialist: Novel mechanism helps, but it does not replace evidence that patients, regulators, physicians, and payers will consider meaningful.

Discovery lead: I will revise the target product profile around the development question.

Development program specialist: Keep the evidence gaps visible so the next study has a clear purpose.

Vocabulary in this exchange: TPP: Target product profile describing desired product attributes and evidence needs.

END REFERENCE

TASK: Extend the conversation library

1. Propose six distinct, common scenarios in this field beyond the supplied case or script. For each, name the setting, two or more professional roles, the communication problem and one useful terminology focus. Include routine coordination, clarification, a complication, disagreement, handoff and follow-up where relevant. Choose scenarios that fit this profession, rather than forcing unsuitable situations into it.
2. Ask me to choose one scenario, or request all six in sequence. Stop and wait. Do not write all the scripts before I choose.
3. For the selected scenario, write an original fictional dialogue of 12-18 substantial speaking turns between two or three professionals. Name each role, establish an actual work problem, and let the exchange progress through questions, clarification, competing constraints and a credible next step or explicitly unresolved issue. Use the occupation's natural nomenclature and register. Avoid an interview between a teacher and a learner, generic small talk, and inserting a glossary definition into every reply. Explain specialist terms outside the dialogue instead.
4. After the script, explain five useful expressions in context, identify two grammar or register choices and ask three questions about the speakers' reasoning. Withhold the answers until I attempt them. Add one role-switch challenge.
5. Before presenting a script, check names, numbers, chronology, roles and terminology for consistency. Label invented facts and acknowledge uncertain specialist usage. If I requested all six, deliver one complete script at a time and wait for "next". Make each scenario materially different.

END PROMPT

Copy this prompt online or choose another lesson and support level